



When Does Federated BatchNorm Help under Differential Privacy?

An Eval-Regime Characterization of DP-FedBN on Medical ECG Classification

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1. Introduction — The BatchNorm-Privacy Tension

- **Federated learning (FL)** trains models across hospitals without sharing patient data; **differential privacy (DP-SGD)** adds a formal guarantee to the updates that are shared.
- **FedBN** keeps each client's **BatchNorm (BN)** statistics local — a per-site domain adapter that is the key to non-IID medical data.
- But BN is **incompatible with DP-SGD**: per-sample gradients are ill-defined, and Opacus's ModuleValidator **refuses** any model containing BatchNorm.
- The standard fix — replace BN with **GroupNorm** — removes the very mechanism FedBN depends on.
- **Question**: can we keep FedBN's personalization **and** satisfy record-level DP — and when does it actually help?

2. Method — DP-FedBN (Dual-Optimizer)

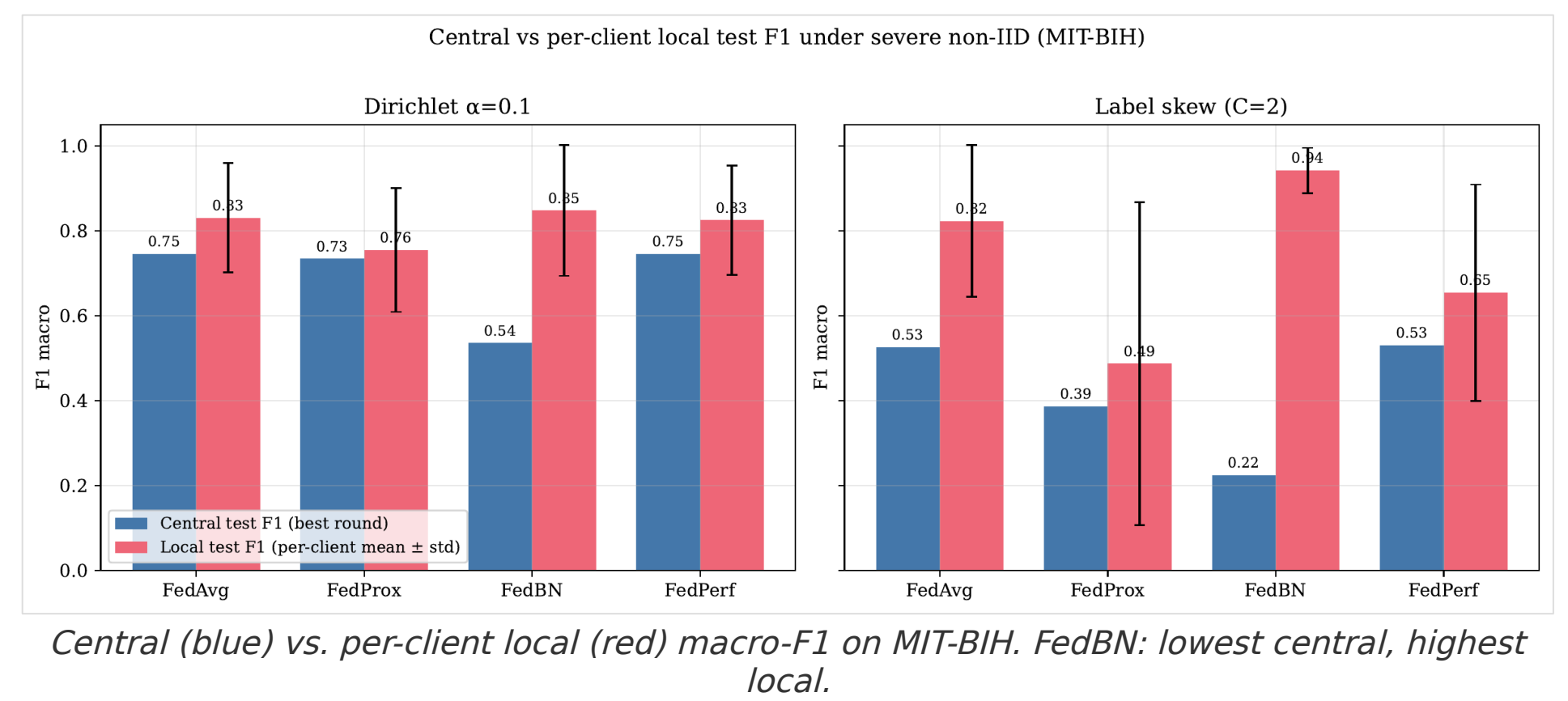
- **Key idea**: BN parameters and running statistics never cross the federation boundary, so they need **no DP**. Apply DP-SGD only to the parameters that are actually shared.
- **Per client, each round**:
 - **Freeze BN** → Opacus's ModuleValidator now accepts the model (freeze-then-unfreeze trick).
 - **opt_A** (AdamW + Opacus PrivacyEngine): non-BN params → per-sample **clipped + Gaussian-noised** (DP-SGD).
 - **opt_B** (plain AdamW): BN params → updated **locally, no DP**.
 - **Server**: FedAvg-average non-BN params; BN stays local (FedBN rule).
- Gives **record-level (ϵ , δ)-DP** on everything that leaves the client while preserving FedBN's per-client BN.
- First **record-level DP-SGD** composition of FedBN with explicit Opacus engineering (vs. the party-level Laplace "DP-FedBN" mention in ADCOL, ICML'23).

3. Experimental Setup

- **Datasets**: MIT-BIH Arrhythmia — 109,453 beats, 5-class AAMI, 360 Hz; PTB-XL — 21,388 records, binary Normal/Abnormal, Lead II, 100 Hz.
- **Model**: 4-block 1D-CNN, ~437k params, 5 BatchNorm modules. K = 5 clients, AdamW, batch 96.
- **Non-IID partitions (6)**: IID, quantity skew, Dirichlet $\alpha \in \{1.0, 0.5, 0.1\}$, label skew C = 2.
- **Privacy**: $\epsilon \in \{1, 3, \infty\}$, $\delta = 1e-5$, clip C = 1.0, Opacus 1.5. Methods: DP-FedAvg (raw BN), DP-FedAvg+GroupNorm, **DP-FedBN**.
- **Two eval regimes**: Central F1 (pooled holdout) vs. Local F1 (per-client). ~40 configurations, single seed (42).

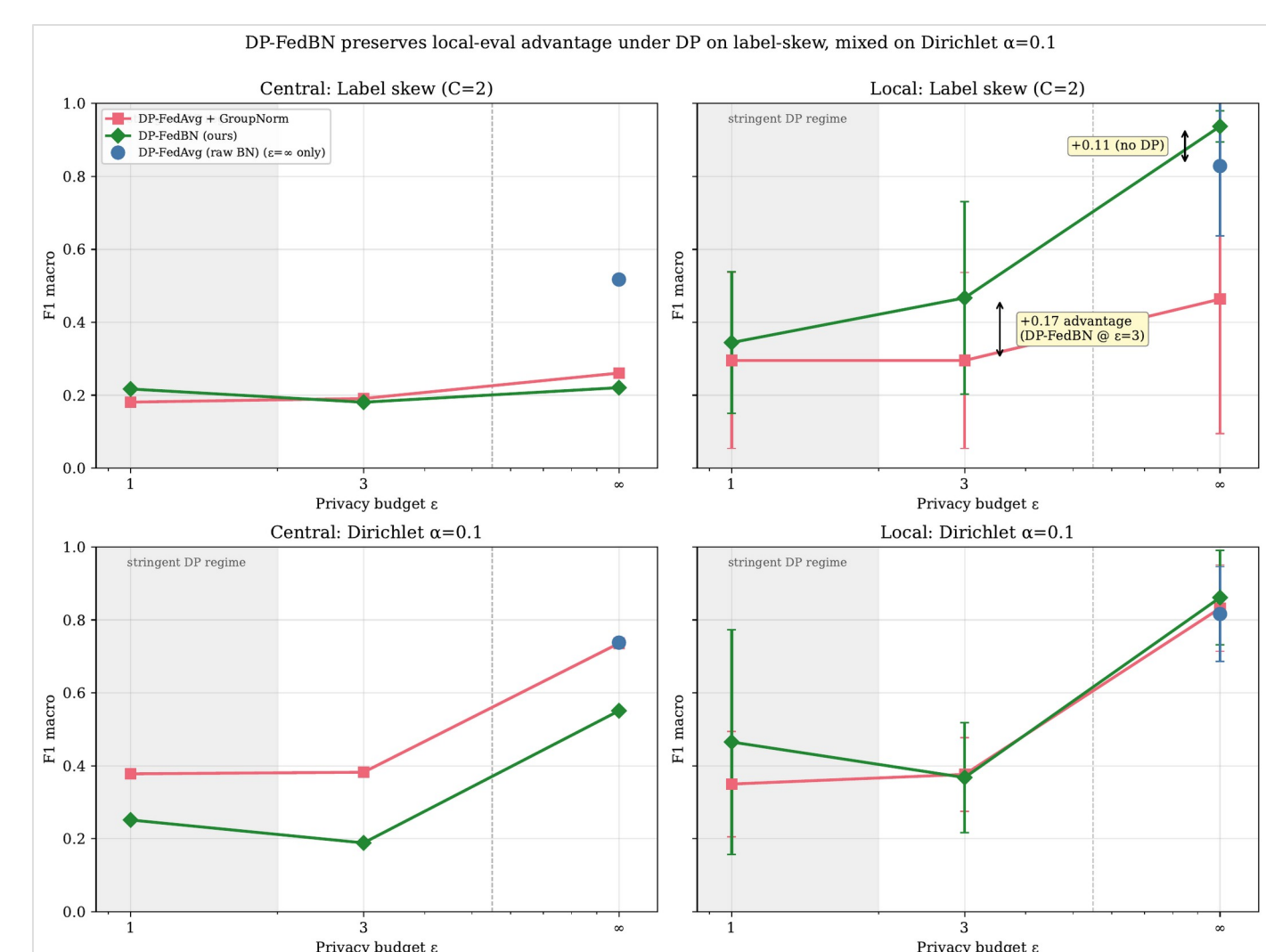
4. Result 1 — The Eval-Regime Flip is Task-Agnostic

- On severe non-IID, FedBN ranks **worst on pooled-central F1** but **best on per-client local F1**.
- MIT-BIH label skew C=2: FedBN central **0.224** (lowest) vs. local **0.942** (highest) — a **+0.72 flip**.
- Replicates on PTB-XL (different task): FedBN–FedAvg = **–0.116 central / +0.123 local** — task-agnostic.



5. Result 2 — Under DP, the Edge is Partition-Dependent

- **Naive DP-FedAvg on a BN model is structurally impossible** — Opacus REFUSES it at every finite ϵ . This motivates DP-FedBN.
- **Label skew** (specialty-clinic regime): DP-FedBN keeps a **+0.17 local-F1 edge** over GroupNorm at $\epsilon=3$ (+0.05 at $\epsilon=1$).
- **Dirichlet $\alpha=0.1$** (proportional skew): edge vanishes/reverses (PTB-XL **–0.234** at $\epsilon=3$) — GroupNorm wins. DP noise hits per-client BN with no cross-client averaging to smooth it.



Privacy-utility frontier (MIT-BIH). DP-FedBN (green) keeps a clear local edge on label-skew at $\epsilon=3$; mixed on Dirichlet.

6. Conclusions & Practitioner Rubric

- The eval-regime flip is **real and task-agnostic** — always report **both** central and local F1.
 - **Rubric**: label-truncated sites (specialty clinics) + local deployment → **DP-FedBN**; proportional case-mix (general hospitals) → **DP-FedAvg+GroupNorm**; strict pooled-central requirement → FedAvg/FedPerf, not FedBN.
 - **Limitations**: single seed, 5 clients, ECG-only, fixed CNN.
- Repro**: github.com/kanemoda/MedicalFL · Opacus 1.5 · PyTorch 2.4.